Healing The Split

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Sonja Stone argues that distinguishing the concept of 'spiritual emergency' from 'failures of ego development', may further stigmatise individuals with psychiatric diagnoses

Transpersonal theory asserts that there are experiences, sometimes referred to as 'spiritual awakening', where the sense of self or identity expands beyond the ego to include all aspects of life, psyche or the cosmos.1 These may present as out-of-body experiences, near-death experiences and visions etc. If a person can retain a grip on consensual reality, even as they open to 'non-ordinary' reality, they may experience a temporary loss of ego-identity, but the ego does not dissolve.2 Occasionally, there is an intensification of the process, which, in extreme cases, may become uncontrollable and terrifying, and be experienced as a 'spiritual emergency'.3

The term 'spiritual emergency' originates from Christina Grof to define and integrate different psychological and transpersonal experiences from many diverse disciplines.4 Grof and Grof define 'spiritual emergency' as psychotic-like states that are critical and experientially difficult aspects of 'spiritual awakening' and transformation.5 When referring to 'spiritual emergencies' or experiences of 'spiritual awakening', accompanied by disturbances and complications, Assagioli stated that 'this happens in those who lack a sound mind, whose emotions are excessive and out of control, in people with too sensitive and delicate a nervous system, or when the flow of spiritual energy has an overwhelming effect due to its suddenness and power'.6

Williams suggests there are two variables which, occurring concurrently, may create optimal conditions for a 'spiritual emergency'.7 The first is a 'spiritual experience' or 'awakening', something that falls outside of consensual reality, or transcends the ego and sense of self, which is difficult to integrate. The second is significant physical, emotional or spiritual stressors, for example, near-death experiences, childbirth, drugs (especially hallucinogenics), emotional deprivation and loss.7

It seems that judgments can be placed on the intensity and significance of the stressor, which may then indicate whether the experience can be classified as a 'spiritual emergency'.

However, I understand that spiritual experiences are subjective and there is no instrument to measure subjectivity; it is indefinable in any physical, material or observable sense.

Classification and measurement of spiritual experiences can diminish and reduce a person's subjective experience into something that is reified and established as a scientific 'truth'.8 By classifying a 'spiritual emergency' using these two variables, there is a danger of supporting a biomedical model of psychosis. The focus for individuals not fitting into the category 'spiritual emergency', may then lean towards their 'deficits' and developmental 'failures' of ego development, without acknowledging any social and environmental aspects of their experiences – past and present.9-11

Socially constructed categorisations of personal experiences can support hierarchical power dynamics in society. This creates fixed boundaries to maintain control over specific marginalised groups, such as those considered as 'mad'.12 When talking about language and terminology in the assessment and diagnosis of psychosis, Dillon says that the subjective opinion about someone else's subjective experience can masquerade as fact.11

Ego development

Childhood or ego development theory is based around the idea of a process towards forming a cohesive sense of 'I' – a conscious and subjective experience of our identity and our selves. Nelson asserts that a child collects and establishes their own share of consciousness from the larger surrounding field, to form a psychic self-boundary, which provides a sense of 'I-ness', the 'me versus not me' essential for sanity in later life.2 These self-boundaries can be tightened, thickened and weakened, through stress and trauma, both physical and psychological. Siegel says that early relational experiences influence wellbeing, social competence, cognitive functioning and resilience in the face of adversity, and that childhood trauma can have long lasting and enduring effects on adult psychological functioning.13

In psychosynthesis theory, the ego or self flows from 'transpersonal Self', which could be perceived as 'a deeper source of wisdom and guidance',14 and which provides continuity to personal being and direction for individual growth and significant engagement with the world. Assagioli saw the 'I-Self' connection as empathic, providing an enduring source of being as 'I-amness'. He suggested that it is through our external empathic connections that our personal self develops, and it is through these that we experience our 'I-Self' connection.15

Meriam suggests that developmental 'failures' of ego development and pathology occur when non-empathic or mis-attuned responses from the caregiver are so overwhelming, at huge odds to the child's needs, and where there is minimal or no reparation available.16 I wonder who or what has failed – the child, the caregiver, the environment, the lack of support, for the caregiver-child dyad? I question whether the 'failure of ego development' could belong to a theory and a society that pathologises, stigmatises and marginalises those who have experienced childhood trauma.

Distinguishing 'spiritual emergency from 'failures' of ego development

How the phenomenon of 'spiritual emergency' is distinguished from 'failures' of ego development has been important to legitimise the concept of 'spiritual emergency' into conventional psychology and psychiatry. Grof4, 5 was keen to make a clear distinction between 'spiritual emergency' and psychosis. He stated that while traditional medical approaches tend to pathologise all 'non-ordinary states of consciousness', there is also a danger of spiritualising and glorifying pathology or overlooking other causes.4

Transpersonal theorists have proposed models to distinguish 'spiritual emergency' from other more 'regressive' states. In these models, a common theme is having enough ego strength to maintain some insight into the process. Assagioli says that it is important that the person can distinguish between the small 'I', the ordinary personality or ego, and the higher 'I' or 'Self'. If this is not recognised, there may be grandiose, absurd and/or potentially dangerous outcomes.6

Lukoff17 provides a detailed framework that assists diagnosis of psychosis with a spiritual potential, so that appropriate support and treatment can be given. He proposes several criteria with which to make the distinction: good pre-episode functioning, acute onset of symptoms with stressful precipitants, and a positive exploratory attitude towards the experience. He says this may also facilitate less medical interventions, such as strong medication, pathological labels and lengthy hospital stays.

In 1994, the category for 'religious or spiritual problem' was included in DSM-IV, which was seen to be a progressive step towards the de-pathologising of 'spiritual emergencies'.18 It seems that this 'progressive step' was supported by an individualist framework that upholds the idea that some 'non-ordinary' states of consciousness have a biological origin or result from psychological distortions, failures and deficits.9 I question, with regards the categorisation of 'spiritual emergency', whether transpersonal theory was also trying to

legitimise itself, and may have fallen foul of a ‘narcissistic grandiosity’ and lost sight of the higher ‘I’ or Self.

Developmental theory suggests that a resilient ego equates with an ability to tolerate all experiences, and that very often this has not been mirrored enough, or at all, in our early environments. It is suggested that those who develop thin or weak self-boundaries can be lose their sense of ‘I-ness’ in intense experiences of ‘non-ordinary’ states of consciousness. Nelson2 hypothesises what can happen in a schizophrenic, ‘non-ordinary’ state. He theorises that the sense of self fragments, making ego boundaries more porous, where the deepest contents of the unconscious empty themselves in the person’s waking life. As the experience intensifies, more threatening archetypal images, which are often diabolical and threatening, overwhelm the person’s experience.

Nelson argues that during ‘non-ordinary’ states of consciousness, the person retreats to lower, more primitive levels of consciousness, and turns away from higher functioning.2 Similarly, Wilber19 separates higher and lower unconscious states: the lower, or pre-personal/pre-egoic, holding childhood and past repressions, and the higher, transpersonal/trans-egoic, holding higher qualities and potential. He characterises human development in terms of hierarchical and successive levels of consciousness. Wilber says his model is not linear, but that each stage must be identified, balanced and coordinated by the self.19 He suggests that the self attempts to balance and integrate everything that is present to the individual; that this balancing act is a key feature of the self, and that psychopathology cannot be understood without it.19 Wilber maintains that most psychopathological typologies can be understood as ‘failures’ of the self’s capacity to differentiate and integrate each level of consciousness.

Going back to Meriam’s16 idea that pathology occurs when non-empathic responses from the caregiver are so overwhelming, and where little or no reparation is available, I question whether this perceived ‘failure’ of integration could also be viewed as a ‘healthy’ response to a set of circumstances and experiences. It reminds me of the concept of homeostasis: ‘the coordinated physiological reactions, which maintain the steady states of the body’.20

Damasio talks about homeostasis as a physiological urge to stay alive, which is engrained in each cell of the body.20 I argue that a ‘failure’ of integration could also be viewed as a resourceful response to potentially life-threatening feelings of annihilation, and I question whether the ‘failure’ of integration could be also re-evoked in a socio-political environment which fails to tolerate and include ‘non-ordinary’ states, such as psychosis, into the

consensual reality. I will explore a client who could be viewed as meeting Grof and Grof's4, 5 and Lukoff's17 criteria for a 'spiritual emergency'.

Case study 1

Terri came for counselling several weeks after giving birth, feeling confused, shocked and distressed. She was unable to sleep or care for her infant, and was scared that she had experienced a post-partum psychosis. Soon after the birth, she had a terrifying experience, which lasted for several days. During this time, Terri felt everything was unreal. She didn't believe the baby was hers or recognise her family. In the assessment, it was clear that Terri had a capacity to reason, she was coherent and keen to try and make some sense of her experience. Our work focused on the impact and the meaning of this episode for Terri.

Jackson21 says there is a need for respectful, sensitive therapeutic interventions, which value any personal meaning of the experiences. Cortright22 suggests that the most powerful intervention in 'spiritual emergency' is education. This reframes and de-pathologises the experience.22 Lucas3 suggests that embodied mindfulness and grounding techniques are key tools in coping with the crisis, as well as assisting in making sense of what has happened, before going back out into the world. I believe that being able to help Terri process the nature of her experience, in small steps, alleviated much anxiety, which, in turn, allowed her to tolerate her experiences. Terri had good support from family and friends and, as the therapy progressed, she accessed this support more and more.

Having reviewed the literature and this client vignette, one could argue that Terri's experience fits into the category of 'spiritual emergency': a 'strong enough ego' with good pre-episode functioning, a significant stressor, and an ability to integrate both 'regressive' and 'progressive' states. From another perspective, however, one could also argue that it was the support of family and friends, who tolerated her experiences, held her safe at home, as well as her ability to access transpersonal counselling so soon after the experience, that contributed to the outcome for Terri. Terri's environment did not 'fail' her. I wonder how she may have managed had she been separated from her family and baby, admitted to a psychiatric unit and given anti-psychotic medication in those first few days – which may have exacerbated her stress and terror and evoked feelings of 'failure'.

I believe that a judgment from a 'professional' on the subjective experience of an individual cannot be neatly conceptualised, and that models such as Nelson's2 and Wilber's19 are

individualist and do not allow for other contexts, such as environmental factors, which can play an important role in creating stressful and traumatic responses to events.

While transpersonal psychology has an epistemology that values many ways of knowing and understanding reality, it has also relied heavily on ‘transcendent’ models of psycho-spiritual development.23 This privileges so called ‘higher’ states, rather than recognising the complexity of all lived experience.23 Clarke24 suggests that the concept of splitting ‘higher’ and ‘lower’ conscious states could be viewed as a way of creating distance from something that is devalued and pathologised by society. She goes on to say that this is a false dichotomy that allows for those in positions of power to say ‘madness is a state very distant from me’ and malign the psychotic experience to the ‘undifferentiated world of the infant’.24

Although transpersonal psychology began a movement towards change in some of psychiatry’s ideas about psychosis, it also created a split between so called ‘spiritual emergencies’ and so called more regressive, negative outcomes such as schizophrenia. I will review another client, who could also be perceived as experiencing a ‘spiritual emergency’, but had a very different experience to Terri.

Case study 2

Kath’s grandmother died very suddenly and shortly after this she began to experience hallucinations. These experiences continued and became more prevalent, during which time she stopped working, was unable to sleep and became very withdrawn. She was admitted into a psychiatric hospital and given a psychiatric diagnosis.

Prior to her grandmother’s death, Kath had held a responsible job and been involved with the community. She also had a complex history of childhood abuse and relationship breakups. Kath felt that when her grandmother died, a lifeline had been cut. I hypothesised that grief ‘loosened’ her ‘self-boundaries’ and that her ‘I-Self’ connection, facilitated by her grandmother, had been threatened with annihilation. I wondered whether she had connected with the hallucinations as a protective mechanism – a way to regain homeostasis – rather like an extreme form of the denial phase of grief, which can feel like a dreamlike state.25

Being given a psychiatric diagnosis and heavily medicated on antipsychotics took its toll and Kath shared her feelings of intense shame and isolation within society and her family dynamics. In the countertransference, her experience in the psychiatric services appeared to reinforce her childhood experiences of ‘empathic failures’ and reinforce the shame of

‘failure’ in relationships. I wonder what could have been different for Kath had she been perceived as experiencing a ‘spiritual emergency’, and had the support of a loving family?

I argue that a culture of pathologising and separating experiences into positive and potentially healing, versus negative and regressive, may possibly repeat and validate a person’s childhood experience and amplify the experience of non-empathic connection. A recent BBC2 Horizon programme26 questioned the social construction of ‘madness’ through the term schizophrenia. It gave voice to three people who had experienced visual and auditory hallucinations and who had all received diagnoses of schizophrenia. What emerged, through their shared experiences, was the idea that the hallucinations provided a ‘route’ for expression of their ‘hidden’ and ‘unvoiced’ childhood trauma, which may be presenting itself for integration. Like Kath, I believe that these people could be viewed as experiencing ‘spiritual emergencies’.

Social constructionism

Transpersonal theorists have used models to categorise those people who have a ‘strong enough’ ego into the category of ‘spiritual emergency’ and exclude those who don’t. The label ‘spiritual emergency’ is socially constructed and can provide protection to people, making the experience more positive, less frightening and facilitating care, which is more empathic and less stressful.27 I believe that, implicit in this idea, is the alternative: that psychiatric intervention is more stressful and frightening and that those who are not given the label ‘spiritual emergency’ are pathologised and given potentially isolating discriminatory labels. This can impact them for the rest of their lives – making it more difficult to have relationships, work and lead a life free from stigma, shame and social exclusion. This can add to their internal and external levels of stress and perhaps trigger further episodes.

Including ‘spiritual emergency’ in DSM-IV was considered a landmark in validating spiritual experience as separate from potentially more destructive processes; but it seems that transpersonal psychology supported and colluded with the biomedical diagnostic belief systems of conventional Western psychiatry.18 Given that the phenomenon of ‘spiritual emergency’ does not acknowledge the socio-political reality of someone’s existence, maybe it’s time transpersonal psychology re-evaluated the concept and added support towards more robust, non-medicalised theories of psychosis. Boyle argues for more sophisticated models of psychosis, which take account of the inseparable, mutual relationships among social, psychological and biological factors, rather than privileging any one of them.9

The term ‘spiritual emergency’ emerged in the 1970s from hierarchical models of human development with influences from Eastern traditions of transcendent growth, at a time when transpersonal psychology claimed a ‘superior grade of reality for the spiritual’.28 I have discovered more contemporary ideas for a 21st century transpersonal psychology, which provide support for my idea that it may be time to review ‘spiritual emergency’. Brooks argues that, in a post-modernist society, transpersonal psychology needs to evolve from its privileging of higher states to include the contextualised, as well as the depth and complexity of all lived experiences.23 She reviews the intersections between feminist and transpersonal psychology and acknowledges the importance of valuing subjectivity ‘to create a new synergistic lived spiritual activism’.23

Daniels considers the absence of ‘real-world’ relevance of transpersonal psychology and suggests that the ‘ascending-individualistic-narcissistic’ agenda, which has dominated the field for so long, needs to be reassessed to stay relevant.29 He discusses the integration of three vectors into transpersonal psychology: ascending (wisdom and faith), descending (psychological integration and hope) and extending (compassion and empathy), which he suggests are all needed to create ‘a truly integral transpersonal perspective’.23 I can see that integrating these concepts may provide a more inclusive perspective and allow for a more contextualised approach towards the complexity of all human experience.

These ideas remind me of Firman and Gila’s14 suggestion that ‘Self’ is present, not only transcendently, in states of ecstasy, but also in the earliest depths of trauma. They say that ‘Self’ can meet us anywhere, and therefore a bridge to ‘Self’ can be built without needing higher conscious qualities or states.14 In addition, they explicitly say that all ‘disorders’ are spiritual in nature, that childhood wounding is spiritual wounding, so that spirit does not need to be included in psychology, as it is already implicitly present.14 I believe that Firman and Gila are presenting a challenge to both transpersonal psychology and mainstream psychology to include all experiences into a consensual reality, and not privilege some over others.

Using these perspectives, I argue for a different conceptualisation of ‘spiritual emergency’, which avoids pathologising those considered to be ‘failing’ in ego development. For example, the phenomenon of ‘spiritual awakening’ could provide a framework for understanding developmental ‘failures’ of ego development. If the idea of ego development were conceptualised as something that constantly shifts in relationship to all inner and outer experiences, then ‘spiritual emergency’ or ‘psychosis’ could provide a lens to view what may

be emerging into consciousness for potential integration. This could also provide a way of understanding a person's resources in the face of adversity, both internal and external, including the circumstances surrounding the onset of the 'emergency', consideration of past experiences, as well as cultural, social and environmental factors in the present. It could open up a more collaborative dialogue between the 'professional' and the 'experiencer', to co-create more helpful, empathic support and 'treatment'.

As a psychosynthesis counsellor, I'm aware of the importance of being able to identify a client's capacity to engage in a psycho-spiritual therapeutic relationship, as well as considering my own strengths and limitations in my capacity to explore some of the extreme experiences that clients might present with. Although I agree with the need to gauge a person's capacity to tolerate any stressful and potentially damaging experience, and provide them with the right support, I also argue that using a dualistic model of identification is flawed, not least because it may rely on hierarchical developmental theories. I argue that integration needs to happen at a societal level; that our cultural 'ego' needs to expand to include all 'non-ordinary' states within the 'ordinary' range of human experience. This could empower the 'experiencers', reducing the 'us versus them' dichotomy, as well as lessening the potential for stigma, shame and social exclusion.

Conclusion

I have explored the phenomenon of 'spiritual emergency' and ways in which it has been distinguished from developmental 'failures' of ego development. There is an implication in some of the literature that people who already have weakened ego boundaries through childhood wounding may experience more regressive or pathological 'non-ordinary' states of consciousness. There is also the idea that it is 'failures' of ego development that facilitate a 'spiritual awakening' to become an 'emergency', to accelerate an expression of the past and create an opportunity for integration. I have argued that it might be impossible to make a distinction, that these concepts are interrelated and they both fail to acknowledge a person's subjectivity and the socio-political reality of their lived experience. I have acknowledged that these concepts and labels are socially constructed and can favour or discriminate certain groups of people, and that new ideas are required to include more contextualised approaches, which value and include the complexity of all human experience.

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